DIGITALAIKART · INDIA EDITION · VERIFIED NOVEMBER 2026

The Indian Blood Pressure Playbook

2026-27

Measure it properly at home. Cut the salt that is hiding in your kitchen. Read your own numbers. Walk into the clinic with a page your doctor can use.

42 pages · 18 chapters · 9 printable worksheets · built on the 2025-26 Indian, WHO and AHA/ACC guidance

DIGITALAIKART
Premium India-specific playbooks · digitalkartai.shop

Read this page first

One in three Indian adults has high blood pressure. Fewer than one in five of them has it under control. Almost nobody has been taught how to measure it at home, what the number should be, or how much of it comes from the salt in their own kitchen. That gap — not the medicine — is where most of the damage happens.

This playbook is not a textbook and it is not a list of foods to avoid. It is a working manual with three jobs: get an honest reading of your blood pressure, find and cut the sodium that is actually in your diet, and turn your next clinic visit into fifteen useful minutes instead of five rushed ones. Everything here is written for Indian kitchens, Indian pharmacies, Indian prices and Indian public health facilities.

How to use it

1. **Week one, do only the audit.** Chapter 4 gives you the seven-day home protocol and Chapter 5 shows you how to read the result. Do not change anything else until you have that number.
2. **Then pick two levers, not seven.** Chapter 6 to Chapter 10 each lower blood pressure by a measurable amount. Two done properly beat seven started.
3. **Print the worksheets.** The log sheets and the doctor summary at the back are the parts that change outcomes. A reading you cannot show your doctor is a reading wasted.
4. **Never change a prescribed medicine on your own.** This book explains your medicines so you can take them correctly and ask better questions — not so you can self-prescribe or stop them.

> **This is education, not a prescription**
>
> Nothing here diagnoses you or replaces your doctor. Blood pressure has to be confirmed by a clinician, and every medicine decision belongs to a registered medical practitioner who has examined you. What this book does is make you a well-prepared patient: correct readings, a real food history, and the right questions. If you are pregnant, on dialysis, or have kidney disease, some advice here changes — Chapter 15 flags exactly where.

What is verified, and how

Every threshold, protocol and price in this book was checked against a primary or institutional source in the first week of November 2026: the Indian Guidelines on Hypertension (IGH)-V 2025-26 published in the Journal of the Association of Physicians of India, the Indian Society of Hypertension consensus guideline 2025, the Indian home blood pressure monitoring guideline published in March 2026, the WHO 2021 guideline for the pharmacological treatment of hypertension, the 2025 AHA/ACC guideline, the National Health Mission's standard treatment guideline and NP-NCD programme documents, and live Indian retail and Jan Aushadhi prices. Where a number can move — drug prices, device prices — the book gives you the range and the date it was checked, and tells you to confirm before you spend.

> **The one number to remember**
>
> Home average of 135/85 mmHg or higher, taken with the protocol in Chapter 4, is high blood pressure. Clinic readings use 140/90. They are not the same number because they are not the same measurement. Mixing them up is the single most common reason people think they are fine when they are not.

Contents

Part One — Your real number

1 What high blood pressure actually is
2 The numbers: clinic, home and 24-hour
3 India's quiet epidemic, in data
4 The seven-day home blood pressure audit
5 How to read your own readings

Part Two — The levers that move it

6 Salt: the 5-gram rule, Indian kitchen edition
7 The plate: potassium, fibre and the DASH idea
8 Movement: walking, strength and the handgrip
9 Weight, waist and the South Asian cut-offs
10 The other levers: alcohol, tobacco, sleep, stress, pills

Part Three — Working with the system

11 Your medicines, without fear
12 The clinic visit: tests, questions, follow-up
13 Red flags and emergencies
14 Special situations: pregnancy, elderly, diabetes, kidney
15 Money, schemes and insurance

Part Four — Doing it

16 The eight-week plan
17 The nine worksheets
18 Sources and verification log

What is in the worksheets

A seven-day home BP log, a thirty-day trend sheet, a salt audit for one week of your own meals, a food-swap card, a medicine and timing card, a doctor-visit summary sheet that fits on one page, a red-flag wallet card, a family history and risk sheet, and an eight-week tracker. All of them are designed to be printed on ordinary A4 paper.

PART ONE

Your real number

Why the reading you got last year is not the reading you should act on

1 · What high blood pressure actually is

Blood pressure is the force of blood against the walls of your arteries. Two numbers describe it. The upper number, systolic pressure, is the peak force as your heart contracts. The lower number, diastolic pressure, is the resting pressure between beats. When that force stays high for months, the damage is mechanical: artery walls thicken and stiffen, the heart muscle grows to push against the resistance, and the fine vessels in the brain, kidneys and eyes take the brunt.

Why it is dangerous precisely because you cannot feel it

Hypertension has no reliable symptom. Headache, neck pain, giddiness and tiredness are not markers of a high reading — they happen at normal pressures too, and plenty of people walk around at 170/105 feeling entirely well. The National Health Mission's own guidance is blunt about this: hypertension is a chronic, persistent, largely asymptomatic disease, and most people who have it in India do not know it. That is why measurement, not symptoms, is how it gets found.

What untreated pressure does, in order

- **Stroke.** Raised blood pressure is linked to more than half of stroke deaths in India and about a quarter of deaths from coronary heart disease.
- **Heart attack and heart failure.** The left ventricle thickens, then stiffens, then struggles.
- **Kidney damage.** High pressure scars the filtering units; protein leaking into urine is the early signal.
- **Eyes.** Hypertensive retinopathy can quietly reduce vision before anything feels wrong.
- **Vessels elsewhere.** Aortic aneurysm, peripheral artery disease, and in men, erectile dysfunction — often the first hint.

> **The good news, stated plainly**
>
> This is one of the few conditions where the arithmetic is genuinely on your side. A sustained 10 mmHg drop in systolic pressure cuts stroke risk by roughly a third and heart failure risk by around a quarter. Some of that 10 mmHg is available from salt alone; more from salt plus walking plus eight kilos of weight. Medicines add the rest. No single step has to be dramatic.

Three things that are not true

- **"There is a normal BP for my age, so 150 is fine at 60."** There is no age-specific normal. The target is the same at 40 and at 70 unless your doctor sets a different one for frailty, and even then it is not 150 by default.
- **"I will take medicine only when the reading is high."** Tablets do not work like a painkiller. They work only while you take them, every day, at the same time. Stopping when the reading looks good is stopping the thing that made it look good.
- **"Once I start, I can never stop."** Some people with genuinely mild hypertension do come off medication after sustained weight loss — but only under supervision, and only after months of home readings. The rule is: never stop on your own, and never assume you are stuck forever.

2 · The numbers: clinic, home and 24-hour

A blood pressure reading is not a property of your body. It is a measurement, taken with a device, in a setting, by a method. Change the setting and you change the number — which is why the same person can be 148/94 in a clinic corridor and 128/82 at their own dining table, twenty minutes later, without anything changing inside them.

The three ways to measure, and what each one is for

Method	What it is	High if	Best used for
Clinic (office)	Two readings, by a trained person, on two visits 1–4 weeks apart	≥140/90 mmHg	Confirming a diagnosis, and the reading your records are built on
Home (HBPM)	Validated upper-arm device, standard protocol, 3–7 days averaged	≥135/85 mmHg	Day-to-day truth, masked and white-coat cases, judging whether treatment works
24-hour (ABPM)	Cuff worn for a full day and night, readings every 15–30 minutes	≥130/80 (24-h average)	Borderline cases, suspected night-time or early-morning surges, resistant pressure

Why the home threshold is lower than the clinic threshold

Clinic measurement overstates the pressure of a nervous patient and understates the pressure of a calm one who is actually running high all day. The Indian home monitoring guideline published in March 2026 sets out how large that gap is in Indian practice: in the India Heart Study, home readings reclassified roughly 24% of people as white-coat hypertension and about 18% as masked hypertension, and more than 40% showed disagreement between the home average and the first clinic reading. In the GRAND study, 21.8% of people being followed up had masked uncontrolled hypertension — normal in the clinic, high at home, and therefore untreated.

Pattern	Clinic reading	Home average	What it means
Normal	under 140/90	under 135/85	Nothing to do but keep an annual check
White-coat hypertension	140/90 or above	under 135/85	The clinic makes you tense. Still needs watching yearly; usually not treated immediately
Masked hypertension	under 140/90	135/85 or above	The dangerous one. You look fine on paper and are not fine. Home monitoring is the only way to find it
Sustained hypertension	140/90 or above	135/85 or above	Real, in both settings. Treat it

Grading, once it is confirmed

Grade	Clinic systolic	Clinic diastolic	What it usually means in practice
Grade 1 (mild)	140–159	90–99	Lifestyle work has real room here; your doctor decides if medicine starts now

Grade	Clinic systolic	Clinic diastolic	What it usually means in practice
Grade 2 (moderate)	160–179	100–109	Almost always needs medicine alongside lifestyle, started promptly
Grade 3 (severe)	180 or above	110 or above	Same-day medical review. If there are symptoms, it is an emergency (Chapter 13)

If you take one thing from this chapter

Buy a validated upper-arm monitor and learn the protocol in Chapter 4. It is the cheapest diagnostic upgrade available to an Indian household — a good device costs about as much as two restaurant dinners, and it tells you the truth about the organ that will otherwise fail silently.

Isolated systolic hypertension, and why it matters after 50

After about 50, the systolic number often climbs while the diastolic number stays normal or even falls, because the large arteries stiffen. A reading of 158/78 is not "fine because the lower number is good". Isolated systolic hypertension is the most common pattern in older Indian adults and it carries the same stroke risk as any other raised pressure. Treat the number that is high.

3 · India's quiet epidemic, in data

India is not on the edge of a hypertension problem. It is in the middle of one, and the numbers have stopped being abstract.

Indicator	Figure	Source and date
Hypertension prevalence, Indian adults	35.5% overall — 38.7% of men, 32.6% of women	ICMR-INDIAB, 2023
Prevalence above age 60	Over 66%	ICMR-INDIAB, as cited in InSH consensus 2025
Raised BP in ages 15–49	About 22.6%, and rising	NFHS-5 (2019–21)
Share with BP controlled below 140/90	15–17.5%	Meta-analyses of 19 and 51 Indian studies
People screened for hypertension under NP-NCD	42.40 crore	MoHFW, as on July 2026
Diagnosed under the programme	7.01 crore	MoHFW, as on February 2026
On treatment under the programme	5.57 crore	MoHFW, as on February 2026
Deaths linked to raised blood pressure	More than half of stroke deaths; about a quarter of coronary deaths	NHM standard treatment guideline

The rule of halves, and why India does worse than half

The old public health shorthand says half of those with hypertension are undiagnosed, half of those diagnosed are untreated, and half of those treated are uncontrolled. India's control rate of 15–17.5% shows the chain breaking earlier and harder. Two consequences follow, and both are practical.

First, the first reading is usually the only reading. Most people meet their blood pressure by accident — an insurance medical, a pre-employment check, a screening camp, a relative's hospital visit. That single reading is often the only data point that exists for the next several years. Second, treatment gets started and then quietly abandoned: a tablet that produces no sensation is a tablet that gets skipped once the fear fades, which is exactly when the pressure returns to its untreated level.

> **What is actually working in the system**
>
> The India Hypertension Control Initiative — run by the health ministry with ICMR and WHO-India — reorganised public clinics around simple drug protocols, free medicine and follow-up registers, and its model is now built into the NP-NCD programme and the NCD portal. If you use a government facility, ask for the hypertension treatment card and the standard protocol. You are entitled to it, and it is the same protocol your private doctor is broadly following.

The risk factors that are genuinely Indian

- **Salt, and where it hides.** Indian intake averages 8–11 g a day against a WHO ceiling of 5 g. Pickles, papad, packaged namkeen, biscuits, bread, sauces, restaurant gravies and street food account for most of the excess — not the salt spoon.
- **Central obesity at a lower BMI.** South Asians develop metabolic risk earlier. Overweight starts at a BMI of 23 and obesity at 25, with waist cut-offs of 90 cm for men and 80 cm for women.

- **Undiagnosed sleep apnoea.** Snoring plus daytime sleepiness plus a thick neck is a common and badly under-recognised cause of stubborn pressure.
- **Cheap painkillers.** Regular NSAID use for knee or back pain raises pressure and blunts the effect of BP medicines.
- **Tobacco in every form, including gutkha and chewing tobacco.** Not just cigarettes.
- **Air pollution.** Long-term PM2.5 exposure raises blood pressure and cardiovascular risk; north Indian winters are the worst window.

Tick the ones that are yours

Each line below changes how hard your doctor should look, and how strict your own target should be. Two or more ticks means a conversation about more than just a tablet.

Risk marker	Why it changes the plan
A parent or sibling with high BP, stroke or heart attack before 55	Family history is one of the strongest predictors; it argues for earlier and tighter control
Waist over 90 cm (men) or 80 cm (women)	Central fat drives insulin resistance, sodium retention and stiffer vessels
Diabetes, or an HbA1c in the pre-diabetic range	Tighter target, and a preference for ACE inhibitors or ARBs
Snoring with daytime sleepiness	Possible sleep apnoea — a treatable cause of stubborn pressure
Regular painkillers for knee, back or joint pain	NSAIDs raise pressure and blunt your medicines
Tobacco in any form, or more than two drinks a day	Both raise pressure directly and multiply cardiovascular risk
High salt household — pickle, papad and namkeen as staples	The single fastest lever you control without a prescription
A reading of 160/100 or more at any time in the past	That is grade 2 pressure and it rarely settles without treatment
South Asian ethnicity with a BMI over 23	Metabolic risk starts earlier; the international cut-offs under-call it

4 · The seven-day home blood pressure audit

This is the most valuable chapter in the book. Done once, properly, it gives you and your doctor a number that is more reliable than anything a single clinic visit can produce — and it takes about ten minutes a day for a week.

Step 1 — Get a device that is worth trusting

You need an automated, upper-arm, oscillometric monitor validated to the AAMI/ESH/ISO standard. Wrist and finger devices are convenient and much less reliable; skip them for diagnosis. The Indian home monitoring guideline is explicit that only validated upper-arm devices should be used, and that they should be recalibrated roughly once a year.

What to buy	Typical Indian price, November 2026	Why
Basic validated upper-arm monitor (single user, memory for last reading)	about ₹1,900–2,000 — for example the Omron HEM-7090 at ₹1,990 recommended retail, or the HEM-7124-IN at ₹1,935 on Tata 1mg	Enough for the protocol. Buy the cuff size that fits your arm
Bluetooth or app-connected monitor (two users, 60-reading memory)	about ₹4,500–4,700 — for example the HEM-7194T1-FLB at ₹4,490 and the HEM-7383T1 at ₹4,610	Worth it if two people in the house are monitoring, or if you will not keep a paper log
24-hour ambulatory monitoring (ABPM)	about ₹1,800–4,000 depending on city, at a diagnostic lab	Only when your doctor needs night-time or 24-hour data

Check the cuff. The standard cuff that ships with most Indian devices fits an arm circumference of 22–32 cm. If your upper arm is larger, you need a large or extra-large cuff — the Indian HBPM guideline gives the sizes: adult 27–34 cm, large adult 35–44 cm, extra-large 45–52 cm. A cuff that is too small reads falsely high, sometimes by 10 mmHg or more. This single error makes more people look hypertensive than almost anything else.

Step 2 — The measurement itself

The protocol below follows the Indian HBPM guideline (March 2026) and the InSH consensus 2025. Follow the order exactly; the small things are the ones that move the number.

1. **Thirty minutes before:** no tea, coffee, cigarette, exercise or heavy meal. Empty your bladder first — a full bladder can add several mmHg.
2. **Sit still for five minutes.** Back supported, feet flat on the floor, legs uncrossed, no talking, no phone in hand.
3. **Arm at heart level**, resting on a table, cuff on bare skin — not over a sleeve. The cuff's lower edge should sit 2–3 cm above the elbow crease.
4. **Take two or three readings, one minute apart.** Ignore the first reading and average the last two. If the first two differ by more than 5 mmHg, take a third.
5. **Write every reading down** with date and time — the ones you do not like as well as the ones you do. Selective recording is a known failure mode and it defeats the whole exercise.

Step 3 — When, and for how many days

Twice a day: morning within one hour of waking, after urinating and before your medicine, breakfast or tea; and in the evening before dinner or before bed. Do this for at least three consecutive days and preferably seven. When you calculate the average, discard day one entirely — first-day readings run high because the process itself is unfamiliar. Two readings per session, two sessions a day, seven days, day one discarded, gives you 24 readings and an average that is close to your true pressure.

Day	Morning (2–3 readings)	Evening (2–3 readings)	Use in average?
Day 1	Yes	Yes	No — discard
Days 2 to 7	Yes	Yes	Yes
Average	Add all usable systolic values, divide by count. Repeat separately for diastolic.		That is your home BP

A worked average, so you can check your own arithmetic

Six days of usable readings, two per session, two sessions a day: systolic values 141, 138, 144, 136, 139, 142, 137, 140, 145, 139, 141, 138 and so on — say 24 values totalling 3,362. Divide by 24: 140.1. Diastolic values totalling 2,112: divide by 24 = 88.0. Your home BP is 140/88. Against the home threshold of 135/85, that is hypertension, and it needs a doctor's review — even if the clinic measured 132/84 the week before.

Step 4 — How often after the first week

For long-term follow-up you do not need seven days every week. Once controlled, one or two days a week is enough, plus a full seven-day block in the week before any clinic visit or medicine change. If your doctor changes your dose, do a fresh seven-day block two to four weeks later — that is the interval at which most BP medicines reach full effect.

Five ways people get a wrong reading

Mistake	Effect on the number	Fix
Cuff over clothing	Can read up to 10 mmHg high	Bare arm, always
Talking during measurement	Raises systolic by several mmHg	Silence for the whole reading
Unsupported back or crossed legs	Raises the reading	Back against the chair, feet flat
Cuff too small for the arm	Reads falsely high	Measure your arm; buy the right cuff
Taking one reading and acting on it	Wildly variable — swings of 20 mmHg within a day are normal	Average 2–3 readings per session, over days

5 · How to read your own readings

You now have a home average. What you do with it depends on which band it falls into, and on whether you already take medicine. Use the tables below as a guide to urgency, not as a diagnosis.

If you do not take blood pressure medicine

Home average	What it means	What to do, and how fast
Under 130/80	Normal range	Recheck with a seven-day block once a year. Keep the log — it is your baseline
130–134 / 80–84	High-normal	Lifestyle work now (Chapters 6–10) and a repeat block in three months
135/85 to 159/99	Hypertension, grade 1 equivalent	See a doctor within two to four weeks with your log. Start the salt and walking work today
160/100 to 179/109	Grade 2 equivalent	See a doctor within a week. Do not wait for symptoms
180/110 or above	Grade 3 equivalent	Same-day medical review. With any of the symptoms in Chapter 13, treat it as an emergency

If you already take medicine

Home average	What it means	What to do
Under 135/85	Controlled	Keep going. Do not stop or reduce on your own. Bring the log to your next review
135–149 / 85–94	Not at target	Show the log at your next visit — within four to six weeks. Ask whether the dose or the timing needs changing
150/95 or above	Clearly uncontrolled	Book an appointment within two weeks. Ask specifically about adherence, timing, salt and painkillers
Under 110/65 with dizziness or falls	Possibly too low	Tell your doctor promptly. This matters most in people over 70

The four questions your average answers

1. **Is this real?** A home average from 24 readings across six days is far harder to fake than one clinic number. If it is high, assume it is high.
2. **Is it the clinic or is it me?** Compare your home average with the clinic reading taken the same week. A big gap in either direction is information, not confusion.
3. **Is the treatment working?** The only honest measure of a BP medicine is the home average four weeks after the change.
4. **Is it getting worse?** A rising trend over three months, with no change in medicine, is a reason to look for a cause — weight, salt, a new painkiller, alcohol, sleep, or a secondary cause (Chapter 14).

> **Do not become a serial measurer**

> The Indian guidance warns about measurement anxiety and about people adjusting their own doses after a single bad reading. Blood pressure swings through the day; a reading of 152/96 at 4 p.m. after a difficult meeting is not a crisis and is not a reason to take an extra tablet. Measure on the schedule, average it, and act on the average with your doctor.

When one reading is wildly out of line — the five checks

1. **Repeat it.** Sit quietly for five minutes and measure again, two readings one minute apart. A single outlier usually disappears on repeat.
2. **Check the cuff.** Bare arm, right size, lower edge 2–3 cm above the elbow crease, tube running down the inner arm and not kinked.
3. **Check your body.** Feet flat, legs uncrossed, back supported, arm at heart level, no talking, bladder empty.
4. **Check the timing.** Tea, coffee, a cigarette, exercise or a heavy meal in the last 30 minutes can each push the number up.
5. **Check the device.** Batteries, tube connections, and whether the cuff is worn out. A device more than a year old without calibration deserves a comparison against the clinic's reading.

What your log should look like when you hand it over

One line per reading: date, time, systolic, diastolic, pulse, and a short note if something unusual happened — missed a dose, slept badly, had a fever, drank alcohol the night before. At the bottom of the page, the six-day average for systolic and for diastolic, written in the same hand. The doctor summary sheet in Chapter 17 does this for you in a single page.

PART TWO

The levers that move it

Six changes that lower blood pressure by measurable amounts — and how to actually do them in an Indian home

6 · Salt: the 5-gram rule, Indian kitchen edition

Salt is the lever with the best return for the least effort, and the one most misunderstood. The target is under 5 grams of salt a day — about one level teaspoon, which is under 2 grams of sodium. Indians average 8 to 11 grams. That means most people need to cut their intake roughly in half, and cutting it in half is worth about 2.6 mmHg of systolic pressure at population level, more in people who are salt-sensitive — which includes a large share of Indians and most people who already have hypertension.

Salt and sodium are not the same number

Packaged food labels in India print sodium in milligrams. Salt is sodium multiplied by 2.5. So 400 mg of sodium is 1 gram of salt. The label rule to memorise: **sodium mg × 2.5 ÷ 1000 = salt in grams.** A packet claiming 800 mg sodium per serving is giving you 2 grams of salt — 40% of your entire day, in one snack.

Label says (per serving)	Salt equivalent	Share of a 5 g day
200 mg sodium	0.5 g	10%
400 mg sodium	1.0 g	20%
800 mg sodium	2.0 g	40%
1,200 mg sodium	3.0 g	60%

Where the salt actually comes from

Not the spoon in the dal. The bulk sits in a handful of items that Indian households use daily without thinking of them as salty. The table below is an audit tool, not a ban list — the point is to see which three items on it are in your house this week.

Item	Why it is heavy	Practical cut
Pickle, achar	Salt is the preservative. Two teaspoons can carry 1–1.5 g	One small piece, twice a week; rinse before serving; never the oil
Papad	Salt plus frying. Two papads can approach 2 g	Roast instead of fry; one papad, not two; not every meal
Packaged namkeen, bhujia, chips	The single largest hidden source for most urban households	Buy the smallest pack, pour a portion into a bowl, put the packet away
Bread, pav, biscuits, rusks	Salt is a dough conditioner; you cannot taste it	Check the label; choose the lower-sodium brand; halve the quantity
Sauces, ketchup, mayo, soy, stock cubes	Concentrated sodium, small volumes, easy to over-pour	Measure with a spoon instead of pouring; use tomato and spices instead
Instant noodles and ready masala mixes	The tastemaker sachet is mostly salt	Use half the sachet; add your own chilli, lime and coriander
Restaurant gravies, Chinese food, street food	Salt plus MSG plus sauces, and you cannot see any of it	Ask for less salt; avoid the gravy-soaked dishes; do not drink the gravy
Cheese, butter, processed meat	Salt for preservation and flavour	Occasional, not daily

Item	Why it is heavy	Practical cut
Salted butter-milk, packaged drinks, soda	Sodium in drinks is invisible	Plain water, unsweetened lassi, fresh lime

The four-week salt cut, done without ruining food

1. **Week 1 — measure.** Buy a 1-gram measuring spoon or use a level teaspoon and a half-teaspoon measure. Cook as usual, but measure the salt you add for a week and write the daily total in the worksheet. Most households discover they use 8–12 g.
2. **Week 2 — cut the added salt by a third.** Take out one-third of the cooking salt. Replace the lost flavour with lime, tamarind, amchur, pepper, jeera, ajwain, garlic, ginger, curry leaves and roasted dhania. Sour and aromatic notes do the work salt was doing.
3. **Week 3 — cut the top three hidden sources.** From the table above, pick the three items you eat most and change the quantity, not the item. Papad from two to one. Pickle from daily to twice a week. Namkeen from a packet to a small bowl.
4. **Week 4 — hold and re-measure.** Keep the new baseline for a full week, then re-take your seven-day BP block. A 5–6 mmHg fall is a realistic result for a household that was genuinely at 10 g.

The salt substitute question — useful, with one real danger

Replacing part of your salt with a potassium-enriched substitute lowers blood pressure and has been shown in a large randomised trial of more than 20,000 people to cut stroke by about 14% and death by about 12%. The usual product is roughly 75% sodium chloride with 25% potassium chloride. The danger: potassium accumulates in kidney disease, and combining a potassium salt with ACE inhibitors, ARBs, spironolactone or potassium-sparing diuretics can push blood potassium dangerously high. If you have any kidney disease, or you take any of those medicines, ask your doctor before you buy one. Otherwise it is one of the cheapest blood-pressure interventions available.

Salt myths worth clearing

- **"Rock salt and pink salt are healthier."** They are still sodium chloride. Trace minerals are irrelevant at the quantity you eat.
- **"If I do not add salt, I am fine."** Added salt is usually less than half your intake.
- **"Sweating it out makes it okay."** Exercise helps pressure through other mechanisms; it does not license a high-salt diet.
- **"My father ate salt all his life."** And the family history of stroke may be exactly why your target is stricter, not looser.

7 · The plate: potassium, fibre and the DASH idea

The DASH pattern — vegetables, fruit, whole grains, pulses, low-fat dairy, nuts, less red meat and sweets — lowers systolic pressure by about 11 mmHg in people with hypertension, and it does so fastest when it is combined with a salt cut. You do not need to eat American food to get the effect. You need the same three mechanics: more potassium, more fibre, less processed food.

Mechanic one: potassium

Potassium helps the kidney excrete sodium and relaxes vessel walls. Indian diets are usually rich in sodium and poor in potassium, which is the worst possible combination. The fix is food, not supplements — potassium tablets are dangerous without supervision.

Food (common Indian portions)	Why it works	How to use it
Coconut water, 1 glass	Potassium-rich, low sodium	One glass after a walk, not the packaged sweetened kind
Banana, guava, papaya, orange, muskmelon	Potassium plus fibre, cheap and seasonal	One fruit at breakfast and one at 4 p.m. — the slot usually filled by biscuits
Dal, rajma, chana, moong, lobia	Potassium, magnesium and fibre together	A katori at lunch and dinner, not just once a week
Palak, methi, bathua, tomato, lauki, bhindi	Potassium and magnesium, low calorie	Two vegetables at lunch and two at dinner is the single biggest change most households can make
Curd, buttermilk (unsalted)	Calcium and potassium	One bowl a day; skip the salt in the chaas
Almonds, walnuts, unsalted peanuts	Magnesium and healthy fat	A small handful, not a bowl

Who must not chase potassium

If you have chronic kidney disease, are on dialysis, or take an ACE inhibitor, ARB, spironolactone or potassium-sparing diuretic, high-potassium foods and salt substitutes can be dangerous. Ask your nephrologist or physician for your own limit before changing anything in this chapter. This is one of the few places in the book where the standard advice is wrong for you.

Mechanic two: fibre and whole grains

Refined flour and polished rice dominate the Indian plate, and both are low in fibre and high in glycaemic load. Swapping half the rice for a millet or a pulse, and half the maida for atta or a millet flour, lowers weight, improves sugar control and moves blood pressure modestly on its own — and considerably when it causes weight loss.

Mechanic three: fewer processed foods

Ultra-processed food is the common thread between high sodium, high sugar, high refined fat and weight gain. A useful rule that requires no counting: if the packet has a health claim on the front and an ingredient list longer than five lines, it is not a staple.

A day that follows all three mechanics

Meal	Plate	Notes
Morning, before tea	Warm water; 5–10 almonds if you are hungry	No biscuits, no rusk, no packaged juice
Breakfast	Two moong or besan chillas / two idlis with sambar / one bowl poha with peanuts and vegetables / two rotis with unsalted curd	Pick one. Add a fruit. Tea or coffee is fine — up to two cups, no added sugar
Mid-morning	One fruit or a glass of unsalted buttermilk	This is the slot where namkeen usually enters
Lunch	One katori dal or rajma, one katori sabzi, one katori curd, two rotis or one katori rice, a salad	Salad first. It fills the plate with potassium and fibre before the grain
4 p.m.	Tea plus roasted chana or a fruit	The danger hour. Decide this in advance
Dinner	Dal or a vegetable with paneer or egg, two rotis, a vegetable, no papad	Finish by 8:30–9 p.m. if you can; late heavy dinners worsen night-time pressure and sleep
Anything salty	One small portion, twice a week	Pickle, papad, namkeen — choose one per day at most, not all three

8 · Movement: walking, strength and the handgrip

Regular aerobic activity lowers systolic pressure by roughly 5–8 mmHg in people with hypertension, and the effect appears within weeks. Strength training adds a few more. The surprise in the recent evidence is isometric work — a static hold, like a wall sit or a handgrip squeeze — which in a 2023 meta-analysis of 270 trials produced the largest average reduction of any mode, around 8 mmHg of systolic pressure, from surprisingly little time.

The walking prescription

What	How much	How hard
Brisk walking	30 minutes, 5 days a week (150 minutes total)	You can talk in short sentences but not sing
Or split it	3 × 10 minutes a day	Same total, easier to fit. Works just as well
Build-up if you are starting from zero	10 minutes daily for week 1, add 5 minutes a week	Stop and rest if you get chest pain, breathlessness at rest, or dizziness
Add stairs or a slope	2 sessions a week	This is where the fitness gain actually lives

The twice-weekly strength session

Two short sessions a week, bodyweight or light weights: sit-to-stand from a chair, wall push-ups, step-ups on a stair, a light dumbbell row with a water bottle, calf raises. Two sets of 10–12 of each. Never hold your breath during a lift — breathe out on the effort. If your pressure is above 180/110, get it down before you start resistance training.

The isometric handgrip, done properly

1. Get a handgrip dynamometer, or use a soft ball you can squeeze to about 30% of your maximum — firm, not painful.
2. Squeeze and hold for 2 minutes. Breathe normally the whole time. Do not clench your jaw or hold your breath.
3. Rest for 1–3 minutes. Repeat. Four holds in total, in one sitting.
4. Do this three days a week, ideally on non-walking days. Total time: about 15 minutes a week.
5. Measure your BP before you start this habit and after four weeks. If it is working, the average will show it.

> **Who should not start an exercise programme without asking first**
>
> If your blood pressure is 180/110 or above, if you have known heart disease, a recent heart attack or stroke, unstable angina, severe valve disease, or you get chest pain or breathlessness on mild exertion — get medical clearance before you begin. For everyone else, walking is safe, and the risk of staying sedentary is far larger than the risk of a brisk walk.

A four-week build-up, if you have not exercised in years

Week	Walking	Add	Total time a week
1	10 minutes a day, 5 days	Nothing else	50 minutes
2	15 minutes a day, 5 days	Two strength sessions, 10 minutes each	95 minutes
3	20 minutes a day, 5 days	Isometric handgrip, 3 days	115 minutes
4	30 minutes a day, 5 days	One hilly route or a stairs session	170 minutes

Two things that quietly cancel the benefit

- **Long sitting blocks.** Eight hours in a chair blunts the effect of a 30-minute walk. Stand up every 45 minutes, even for two minutes.
- **Exercising and then eating it back.** A 30-minute walk burns roughly what one samosa and a sweet tea replace. Movement is for the vessels, not a licence for the snack.

9 · Weight, waist and the South Asian cut-offs

Weight loss lowers blood pressure by roughly 1 mmHg for every kilogram lost, and the effect is larger when the weight comes off the abdomen. For an Indian adult carrying 8 extra kilos around the middle, that is a realistic 8–10 mmHg — comparable to a full dose of a single medicine, and it improves sugar, lipids, sleep and knee pain at the same time.

Use the South Asian numbers, not the international ones

Measure	Normal	Overweight	Obese	Notes
BMI (kg/m²), South Asian cut-off	18.5–22.9	23.0–24.9	25.0 and above	The international 25/30 cut-offs under-call risk in Indians
Waist, men	under 90 cm	90 cm or more	—	Measure at the navel, standing, after breathing out. Not the trouser size
Waist, women	under 80 cm	80 cm or more	—	Same method

Why the waist matters more than the weighing scale

Visceral fat — the fat around the organs — is metabolically active and drives insulin resistance, which drives sodium retention and stiff vessels. Two people can weigh the same and carry very different risk. Track your waist once a month, in the same place, with the same tape, and you will see change before the scale shows it.

A weight plan that survives an Indian household

- **Fix the liquid calories first.** Sweetened tea three times a day, packaged juice, cold drinks and evening alcohol are the easiest 300–500 calories to remove without any hunger.
- **Change the ratio, not the cuisine.** Half the plate vegetables and dal, a quarter grain, a quarter protein. Same food, different proportion, and no separate cooking for you.
- **Eat protein at breakfast.** Eggs, paneer, besan, moong, curd. A protein-light breakfast is what makes 11 a.m. biscuits feel necessary.
- **Do not drink your fruit.** Whole fruit gives fibre and satiety; juice gives sugar.
- **Aim for 0.5 kg a week.** That is 2 kg a month, 6 kg in three months, and 6 mmHg. Faster loss is usually regained.

> **If you are on BP medicine and lose weight**
>
> Losing 5–10 kg often brings pressure down enough that a dose becomes too high — which shows up as dizziness on standing, tiredness, or home readings under 110/65. Do not reduce the tablet yourself. Book a review with your log, and let your doctor adjust. This is a good problem, and it is one of the few situations where the medicine genuinely can be reduced.

10 · The other levers: alcohol, tobacco, sleep, stress, pills

These five are where stubborn blood pressure usually hides. Each is worth a few mmHg on its own; together they are often the difference between a controlled patient and a "resistant" one.

Alcohol

Alcohol raises blood pressure in a dose-dependent way, and cutting back lowers systolic pressure by about 4 mmHg in regular drinkers. The practical version: no more than two standard drinks a day for men and one for women, several alcohol-free days a week, and none at all if your pressure is uncontrolled. One standard drink is 30 ml of spirit, 100 ml of wine, or 330 ml of regular beer — not a large peg poured by eye.

Tobacco — every form

A cigarette raises pressure for 20–30 minutes; doing it ten times a day means your vessels spend much of the day under strain. Chewing tobacco and gutkha carry the same cardiovascular risk and an additional cancer risk, and are far more common in India than smoking. Quitting is the single highest-value change in this book for anyone who uses tobacco, and pressure starts improving within weeks. Nicotine replacement and counselling through a cessation clinic work better than willpower alone.

Sleep, and the apnoea question

Short sleep and poor sleep quality raise blood pressure; seven to eight hours is the target. The bigger issue is obstructive sleep apnoea, which is common, badly under-diagnosed in India, and a classic cause of pressure that will not come down. Ask yourself the STOP-BANG questions:

Letter	Question	Score 1 if yes
S	Do you snore loudly — louder than talking, heard through a closed door?	
T	Do you often feel tired or sleepy during the day?	
O	Has anyone observed you stop breathing or gasp during sleep?	
P	Do you have high blood pressure, or are you treated for it?	
B	Is your BMI above 35 (or above 30 with South Asian cut-offs)?	
A	Are you over 50?	
N	Is your neck circumference large — over 40 cm?	
G	Male?	

Three or more yes answers, together with pressure that will not come down on two or three medicines, is a reason to ask your doctor for a sleep study. Treating apnoea can drop pressure substantially and improve daytime energy at the same time.

Stress, and what actually helps

Chronic stress raises pressure through sustained sympathetic activation, poor sleep, more salt, more alcohol and less movement. Slow breathing — six breaths a minute for ten minutes, once or twice a day — has modest but real effects, as do yoga and mindfulness-based programmes. What does not work is treating stress as the sole explanation for a 165/100 reading. Stress management is an add-on

to measurement, salt, movement and medicine; it is not a substitute.

The pills and powders that raise your blood pressure

What	Why it matters	What to do
Painkillers: ibuprofen, diclofenac, naproxen, and combination cold remedies containing them	Raise pressure by several mmHg and blunt ACE inhibitors, ARBs and diuretics. Regular use for knee or back pain is a common cause of "resistant" hypertension	Ask about paracetamol instead; use the lowest dose for the shortest time; tell your doctor you take them
Decongestant nasal drops and tablets (xylometazoline, pseudoephedrine, phenylephrine)	Constrict vessels; raise pressure and pulse	Use for no more than 3 days; avoid if pressure is uncontrolled
Licorice / mulethi, in sweets, teas and cough syrups	Causes sodium retention and potassium loss; a recognised cause of high pressure	Check labels; avoid regular use
Steroids	Raise pressure and sugar	Never stop on your own; ask your doctor to monitor pressure during a course
Excess caffeine and energy drinks	Short-term rises of several mmHg; energy drinks more so	Two cups of tea or coffee a day is fine for most people
Some supplements and "fat burners"	Unregulated, often stimulant-based	Stop anything that promises energy or thermogenesis

Bring a list, not a memory

Before your next appointment, write down every tablet, powder, syrup, supplement and ayurvedic preparation you have taken in the last month — including the ones bought without a prescription. The worksheet in Chapter 17 has the format. This list is often the fastest route to explaining stubborn pressure.

PART THREE

Working with the system

Your medicines, your clinic, your emergencies, your money

11 · Your medicines, without fear

Blood pressure medicine is one of the most studied classes of drug in existence. The goal of this chapter is not to make you a prescriber — it is to make you a patient who takes the right tablet at the right time, recognises the two or three side effects that have easy fixes, and knows which combinations are standard so that nobody can sell you an unnecessary one.

The three families, and what they do

Class	Examples (generic names)	How it works	Typical side effects to know
ACE inhibitor	Ramipril, enalapril, lisinopril	Relaxes vessels by blocking angiotensin	Dry cough in some people — the usual reason to switch to an ARB
ARB (sartan)	Telmisartan, losartan, olmesartan	Same pathway, different door; no cough	Dizziness early on; potassium and creatinine need periodic checks
Calcium channel blocker	Amlodipine, nifedipine (long-acting)	Relaxes vessel muscle	Ankle swelling, flushing, headache — often dose-related and manageable
Thiazide / thiazide-like diuretic	Chlorthalidone, indapamide, hydrochlorothiazide	Removes extra sodium and water	Low potassium, raised uric acid and sugar; gout can flare
Beta blocker	Metoprolol, atenolol, bisoprolol	Slows the heart, lowers output	Tiredness, cold hands, vivid dreams. Never stop abruptly

Why single-pill combinations are standard now

The WHO guideline recommends starting with a combination, preferably as a single pill, for most people who need medicine. Two low-dose drugs from different families control pressure better with fewer side effects than one drug at high dose, and one pill once a day is easier to actually take. Telmisartan with amlodipine is the most common Indian combination, and it is on the WHO essential medicines list as a class. If you are on two or three separate tablets, ask whether a single-pill combination exists — it usually does, and it usually costs less.

What these medicines cost, honestly

Prices below were checked in November 2026. The gap between a branded strip and the government's Jan Aushadhi price is the single biggest controllable cost in Indian hypertension care.

Medicine (per 10 tablets unless stated)	Branded, approximate	Jan Aushadhi, approximate	Monthly cost at one tablet a day
Telmisartan 40 mg	₹98 (Telma 40)	₹12	₹36 at Jan Aushadhi vs about ₹294 branded
Amlodipine 5 mg	₹45 (Amlong 5)	₹4.50	₹14 vs about ₹135
Telmisartan 40 + amlodipine 5 (single pill)	₹135 (Telma AM)	₹14.50 per 10, or ₹22.69 per strip of 15 under PMBJP drug code 417	About ₹45–68 vs about ₹405

Three things to say to your pharmacist and doctor about cost. First, ask for the generic name, not the brand. Second, ask whether the same salt is available at the Jan Aushadhi Kendra nearest you — the scheme has thousands of outlets and a fixed printed MRP that is the same everywhere. Third, never

stretch a strip to save money by taking a tablet every other day; a half-treated pressure is still doing damage, and the stroke you are trying to prevent costs more than a decade of tablets.

Adherence: the five reasons people miss doses

1. **No symptom, no reminder.** Link the tablet to something that happens every day without fail — brushing teeth, the first cup of tea, the morning puja. Put the strip next to it.
2. **Running out.** Refill when you have seven days left, not when the strip is empty. Keep a spare week at home.
3. **Side effects nobody explained.** Ankle swelling and cough have straightforward fixes; say the words rather than silently stopping.
4. **Cost.** See the Jan Aushadhi table above.
5. **Fear of "getting addicted".** These medicines do not create dependence. They replace a control mechanism your body is not providing on its own.

> **Never stop suddenly**
>
> Stopping a beta blocker or a central-acting drug like clonidine abruptly can cause a dangerous rebound rise in pressure. Stopping anything else simply returns you to untreated levels, which feels like nothing until it isn't. If you want to stop or reduce, do it as a planned change with your doctor, with home readings before and after.

Timing, and the evening question

Take your medicine at the same time every day, generally in the morning, unless your doctor has specifically advised evening dosing. The large "take it at night" trials have not shown a consistent advantage, and evening dosing increases the risk of night-time low pressure and falls in older people. What matters more than the hour is consistency and not missing doses.

12 · The clinic visit: tests, questions, follow-up

A fifteen-minute consultation cannot cover this condition unless you arrive prepared. The single most useful thing you can carry into the room is a one-page summary of your home readings — and the second is a written list of what you actually take.

What a proper first assessment includes

Test or check	Why it is done	Rough Indian cost if done privately
Creatinine with eGFR	Kidney function — both a cause and a casualty of high BP	₹150–350
Serum sodium and potassium	Baseline before starting ACE inhibitors, ARBs or diuretics	₹200–500
Fasting glucose and HbA1c	Diabetes and hypertension travel together	₹150–600
Lipid profile	Total cardiovascular risk, not just pressure	₹300–800
Urine albumin-to-creatinine ratio	Earliest sign of kidney damage from pressure	₹300–800
ECG	Left ventricular thickness, rhythm, old damage	₹200–600
Fundus examination, where available	Hypertensive changes in the retina	₹300–800

These are not all needed on day one in every setting — the WHO guideline is clear that testing should not delay starting treatment. But within the first few months, this is the reasonable set, and it is the same set a public NCD clinic will do.

Eight questions worth asking

1. What is my home average telling you, and what is my target number?
2. Is this the new or the old pattern — and do I have any sign of damage to heart, kidney or eyes yet?
3. Which medicine is this, what is the generic name, and why this one for me?
4. What side effects should make me come back rather than stop the tablet?
5. When should I repeat the home readings, and when do you want to see the log again?
6. Do I need a sleep study, given my snoring and daytime sleepiness?
7. Is anything else I take — painkillers, cold remedies, supplements — pushing my pressure up?
8. Can this be treated with a cheaper generic or a single-pill combination without losing anything?

Follow-up intervals that reflect the guidance

After starting or changing a medicine: review in about a month, and keep reviewing monthly until you are at target. Once controlled: every three to six months, with home monitoring in between. Once a year: the full review — kidney function, electrolytes, lipids, sugar, weight, waist, medicine list, and side effects. If your pressure is not at target on three medicines including a diuretic, that is the definition of resistant hypertension and it deserves a proper look at adherence, salt, painkillers, sleep apnoea and secondary causes.

If the appointment is three weeks away

- **Start the lifestyle work now.** Salt and walking do not need a prescription, and the eight-week plan in Chapter 16 is designed to run in parallel with waiting.
- **Keep measuring.** Three weeks of logs is not wasted time — it is the best evidence you will bring.
- **Use a teleconsultation for the interim question.** Free national teleconsultation through eSanjeevani, or your insurer's app, can review a log and tell you whether to come in sooner.
- **Go sooner if you cross a red line.** 180/110 or above, any symptom in Chapter 13, or a reading above 160/100 with diabetes or kidney disease — do not wait for the appointment.

Changing doctors, or wanting a second opinion

There is nothing wrong with asking for one, and it is not an insult to your physician. Ask for a copy of your reports and your prescription, carry the home log, and state the question plainly: is my pressure at target, and if not, why not? A second opinion is most useful when it is specific — about a drug choice, a suspected secondary cause, or a reading that does not match how you feel.

> **Use what you already pay for**
>
> Free teleconsultation through eSanjeevani connects you to a doctor without a journey, and your ABHA health account keeps your records together across facilities. If you use a government Ayushman Arogya Mandir or PHC, the hypertension medicines on the standard protocol are supplied free — ask for the treatment card, and carry it to every visit.

13 · Red flags and emergencies

Most high readings are not emergencies. A few are. The distinction is not the number alone — it is the number plus the symptoms.

Call for help immediately if a very high reading comes with any of these

Chest pain or pressure; severe breathlessness; weakness or numbness on one side of the face, arm or leg; slurred speech; sudden loss of vision or double vision; severe headache with vomiting; confusion or drowsiness; a seizure; or a reading of 180/120 or above with any of the above. This is a hypertensive emergency or a stroke until proven otherwise. Do not wait for the reading to come down at home.

Stroke: the test anyone can do in 30 seconds

Letter	Ask the person to	Warning sign
F — Face	Smile	One side of the face droops
A — Arms	Raise both arms and hold	One arm drifts down or cannot lift
S — Speech	Repeat a simple sentence	Slurred, wrong words, cannot speak
T — Time	Note the time symptoms began	Every minute costs brain. Get to a hospital with stroke care now

Time is the treatment. Clot-dissolving and clot-retrieval treatment works within a narrow window from symptom onset, so the single most useful fact you can give the hospital is the exact time the symptoms started. Note it, do not estimate it later.

Very high without symptoms

A reading of 180/110 or more with no symptoms at all is usually called hypertensive urgency, not emergency. It still needs a doctor the same day — not a panic, and not a double dose. Sit quietly for five minutes and repeat the measurement, because a single very high reading is often a measurement artefact or a stressful moment. If it stays that high on repeat, get medical advice the same day.

Five things not to do

- **Do not take an extra tablet** to bring a number down quickly. Nothing you have at home is designed for that, and the drop can overshoot.
- **Do not put a tablet under the tongue** — a habit borrowed from an outdated practice that can drop pressure dangerously.
- **Do not lie down flat and wait** for stroke symptoms to pass. Go to a hospital.
- **Do not drive yourself** with chest pain, breathlessness or neurological symptoms.
- **Do not stop all your medicines** after one bad reading. Bring the log to a doctor instead.

What to carry, always

A wallet card with your diagnoses, your medicine list with doses, your allergies, your doctor's number and one emergency contact. The card in Chapter 17 is sized to cut out and keep. In an emergency, the person helping you may not be able to ask you anything.

14 · Special situations

The standard advice changes in five situations. If any of them is you, read this page before acting on the rest of the book.

Pregnancy

Blood pressure is measured and treated differently in pregnancy. Thresholds for starting medicine are higher, and some of the commonest drugs — ACE inhibitors and ARBs — are harmful to the foetus and must be stopped before conception or as soon as pregnancy is confirmed. Pre-eclampsia is a distinct and dangerous condition with headache, visual disturbance, swelling and protein in the urine. Home monitoring is feasible and useful in pregnancy but the decisions belong to an obstetrician. Do not use any advice in Chapters 6–11 to manage pregnancy-related blood pressure on your own.

Older adults, especially over 70

- **Standing pressure matters.** Measure once after sitting five minutes, then again one minute after standing. A big drop with dizziness means the medicine may be too strong, or the timing wrong.
- **Falls are the real risk.** A pressure that is too low causes falls, fractures and loss of independence — sometimes worse than a mildly high reading.
- **Frailty changes the target.** For a frail 85-year-old with multiple conditions, a doctor may accept a higher target. That is a clinical judgement, not a licence to ignore 170.
- **Dehydration is seasonal.** Summer heat and diarrhoea on a diuretic can crash the pressure. Ask what to do if you cannot eat or drink normally.

Diabetes

The target is usually tighter — a systolic below 130 is commonly suggested for people with diabetes and high cardiovascular risk — and the drug choice usually includes an ACE inhibitor or ARB because of the kidney-protective effect. Home monitoring is particularly valuable here, because pressure swings are wider.

Chronic kidney disease

Two things change immediately. Potassium-rich foods and potassium-based salt substitutes can be dangerous, and the choice of medicine is closely tied to kidney function. Do not adopt Chapter 6's salt substitute or Chapter 7's potassium list without your nephrologist's agreement. Everything else in the book still applies.

Young hypertension, under 40

High pressure in a young adult deserves a search for a secondary cause rather than a lifelong prescription handed over in five minutes. Ask specifically about: kidney disease, a narrow or blocked renal artery, thyroid disease, hormonal causes such as Conn's syndrome or phaeochromocytoma, and obstructive sleep apnoea. Also check for white-coat effect — a young person's first clinic reading is a poor basis for a lifetime diagnosis.

White-coat and masked cases in one line each

White-coat hypertension: high in clinic, normal at home — usually not treated immediately, always reviewed yearly, because a share of these people develop sustained hypertension. Masked hypertension: normal in clinic, high at home — needs treatment, and is found only if somebody measures at home.

15 · Money, schemes and insurance

Hypertension is cheap to treat and expensive to ignore. The treatment cost is a few hundred rupees a month at most; the cost of a stroke is a hospital bill, lost income and often a family member who stops working to provide care.

Cost head	Realistic Indian range, November 2026	How to reduce it
Validated home BP monitor, one-time	₹1,900–4,700	Basic upper-arm model is enough; one device serves the household
24-hour ABPM, one-time test	₹1,800–4,000 depending on city	Ask whether home monitoring would answer the same question first
Medicines, per month	₹15–70 at a Jan Aushadhi Kendra; ₹135–405 branded	Ask for the generic name; use Jan Aushadhi where available
Annual review tests	₹1,500–3,500 privately; free or near-free at a public facility	NP-NCD screening at an Ayushman Arogya Mandir or PHC
Consultation	Free at a public facility; ₹300–1,000 privately	eSanjeevani teleconsultation is free

The schemes worth knowing by name

- **NP-NCD population-based screening.** Everyone aged 30 and above is eligible for free screening for hypertension, diabetes and three common cancers at an Ayushman Arogya Mandir, PHC or sub-centre. By July 2026, 42.4 crore people had been screened for hypertension under the programme.
- **The NCD portal and your treatment card.** Once diagnosed under the programme you get a treatment card, free protocol medicines and follow-up through ASHA and ANM workers. Ask for it — it exists whether or not anyone offers it.
- **Ayushman Bharat PM-JAY.** Covers hospitalisation at secondary and tertiary level for eligible families. It does not cover routine outpatient medicine or consultations; a parliamentary committee has recommended adding OPD cover for chronic diseases, but that is a recommendation, not yet a benefit. Do not assume your monthly tablets are covered.
- **eSanjeevani.** Free national teleconsultation. Useful for follow-up and for reading a home log with a doctor.
- **ABHA.** A health account that keeps your records in one place across facilities — worth creating before you need it.

The insurance mistake that costs families everything

If you buy a health policy or a critical illness cover, declare your hypertension and your medicines truthfully at the proposal stage. A claim for a stroke or heart attack is exactly where an insurer will look for an undisclosed pre-existing condition. Declaring it may raise the premium slightly or add a waiting period; not declaring it can void the claim entirely. If you already have a policy and have never mentioned your blood pressure, ask the insurer in writing how to disclose it now.

PART FOUR

Doing it

An eight-week plan, nine printable worksheets, and the sources behind every number

16 · The eight-week plan

Eight weeks is enough to know whether the lifestyle levers are working for you, and enough to build the habits that make them permanent. Do not attempt everything in week one. The order below is deliberate: measure first, then cut salt, then move, then review.

Week	The one job	Also	What you should have at the end
1	Buy a validated upper-arm monitor and learn the protocol	Print the seven-day log	A device, a cuff that fits, and confidence in the method
2	Run the full seven-day audit	Discard day 1; average days 2–7	Your real home BP number
3	Start the salt audit and cut added salt by a third	Add lime, tamarind and spices back in	A measured daily salt figure
4	Cut your three biggest hidden salt sources	Walking: 10 minutes a day	A shopping list that has changed
5	Walking 30 minutes, 5 days a week	Two strength sessions	The first week where exercise happened every day
6	Fix the plate: two vegetables twice a day, one fruit twice a day	Isometric handgrip, 3 days a week	A plate that looks different, not a diet you are enduring
7	Review your medicines, side effects and cost	Book the clinic visit; fill the doctor summary sheet	A one-page summary and a list of questions
8	Repeat the seven-day audit	Compare with week 2	A measured result, and a decision with your doctor

What a good eight weeks looks like in numbers

Salt down from about 10 g to about 5–6 g: roughly 3–5 mmHg. Walking 150 minutes a week: 5–8 mmHg. Three to four kilos of weight loss: 3–4 mmHg. Alcohol cut or stopped: about 4 mmHg. Taken together, a realistic combined fall for someone who does all four is 10–15 mmHg of systolic pressure — which is the same order of magnitude as adding a medicine, without the cost or the side effects.

If nothing moves

If your average has not shifted after eight weeks of honest effort, that is useful information, not failure. It usually means one of five things: the salt audit under-counted restaurant and packaged food; you need medicine and there is no shame in that; there is a secondary cause such as sleep apnoea; something you take — a painkiller, a cold remedy, licorice — is pushing it up; or the measurements themselves are unreliable. Take the log and the honest food history to your doctor and work through those five in order.

17 · The nine worksheets

Print these pages. They are deliberately plain: black text on white, no shading that costs ink, and enough space to write with a pen. A log you keep on a phone is fine too, as long as you show it to your doctor.

Worksheet 1 — Seven-day home blood pressure log

Two sessions a day, 2–3 readings per session, one minute apart. Write every reading, including the ones you do not like.

Day	Morning 1	Morning 2	Morning 3	Evening 1	Evening 2	Evening 3	Notes (missed dose, poor sleep, alcohol)

Average of days 2–7 — systolic: _____ diastolic: _____ readings used: _____

Worksheet 2 — Thirty-day trend sheet

Once you are at target, one or two days a week is enough. One line per entry keeps the trend visible between clinic visits.

Date	Morning average	Evening average	Pulse	Tablet taken?	Notes

Month summary — average systolic _____ average diastolic _____ days missed: _____

Worksheet 3 — One-week salt audit

Measure added salt with a level teaspoon (about 5 g). Then estimate the salt from the packaged, pickled and restaurant items you ate.

Day	Salt added in cooking (g)	Packaged and pickled items (g est.)	Restaurant and street food (g est.)	Total (g)	Notes

Weekly total: _____ g Daily average: _____ g Target: under 5 g a day

Worksheet 4 — Food swap card

Tick the swaps you will actually make this month. Three is enough to start.

	From	To
	Two papads with dinner	One roasted papad, three times a week
	Packet of namkeen in the evening	Roasted chana or a fruit
	Pickle at every meal	Pickle twice a week, one small piece
	White bread or pav at breakfast	Poha, upma, idli or two rotis
	Sweetened tea three times a day	Unsweetened tea, or two cups with less sugar
	Cold drink or packaged juice	Water, unsalted buttermilk, fresh lime
	Restaurant gravy dish every week	Dry or tandoori dish, once a fortnight
	Biscuits with evening tea	Almonds, walnuts or roasted chana
	Salt in the dal plus salt at the table	Salt only in cooking, measured
	Late heavy dinner	Dinner by 8:30–9 p.m., lighter

Worksheet 5 — Medicine and timing card

Fill this once and keep a copy in your wallet. Carry the generic names, not just the brands.

Medicine (generic name)	Strength	When	With food?	Since when	Prescribed by

Worksheet 6 — Doctor visit summary — one page

Fill this before every appointment and hand it over at the start. It saves five minutes and gets you a better answer.

Name, age, sex	
Known conditions	
Home BP average (with dates)	___ / ___ from ___ readings, days ___ to ___
Clinic BP today	
Medicines and doses	
Side effects noticed	
Other tablets, syrups, supplements, painkillers in the last month	
Salt: estimated daily intake	___ g
Weight and waist	___ kg, ___ cm
Exercise in a typical week	
Sleep, snoring, daytime sleepiness (STOP-BANG score)	___ / 8
Alcohol and tobacco	
My three questions	1. 2. 3.

Worksheet 7 — Red-flag wallet card

Cut along the border and keep this in your wallet or purse.

IF A VERY HIGH READING COMES WITH ANY OF THESE, GET EMERGENCY HELP NOW

Chest pain or pressure · severe breathlessness · weakness or numbness on one side · slurred speech · sudden loss of vision · severe headache with vomiting · confusion or drowsiness · seizure

Stroke test — F.A.S.T. Face droop · Arm drift · Speech slurred · Time: note when it started, then go now

My details

Name: _____ Blood group: _____

Conditions: _____

Medicines: _____

Allergies: _____ Doctor: _____

Emergency contact: _____

Worksheet 8 — Family history and risk sheet

Fill this once. It changes what your doctor should be looking for, and your children will need it one day.

Relative	Condition (BP, stroke, heart attack, diabetes, kidney)	Age at diagnosis	Notes

Also note: your own age at the first high reading, any pregnancy-related high blood pressure, and whether you have ever had a kidney problem or a kidney test that came back abnormal.

Worksheet 9 — Eight-week tracker

One tick per day. Do not aim for a perfect row — aim for more ticks than last week.

Week	Measured BP	Salt under 5 g	Walked 30 min	Strength or grip	No packaged snack	Slept 7 hours	Weigh t

At the end of week 8 — systolic change _____ diastolic change _____ kilos lost _____ waist change _____

What to do with these pages

Keep the log sheets in a folder with your prescriptions. Take the doctor summary sheet, the medicine card and the last four weeks of logs to every appointment. Take the wallet card everywhere. Nothing else in this book works as well as a page your doctor can read in ten seconds.

18 · Sources and verification log

Every clinical threshold, protocol and price in this book was checked in the first week of November 2026. Where a figure moves — drug prices, device prices, retail offers — the range and the check date are stated in the text, and you should confirm before spending. This log exists so that you, or your doctor, can go to the source rather than take the book's word for it.

Clinical guidance and programme documents

- Indian Guidelines on Hypertension (IGH)-V, 2025-26 — Journal of the Association of Physicians of India, published May 2026.
- Indian Society of Hypertension (InSH) Consensus Guideline for the Management of Hypertension, 2025 — including the home blood pressure monitoring protocol, the discard-day-one rule and the cuff size table.
- Indian home blood pressure monitoring guideline, March 2026 — device validation (AAMI/ESH/ISO), the two-sessions-a-day protocol, diagnostic thresholds, and the India Heart Study and GRAND study findings on white-coat and masked hypertension.
- WHO, Guideline for the pharmacological treatment of hypertension in adults, 2021 — treatment thresholds, first-line drug classes, single-pill combination preference, targets (<140/90 generally, systolic <130 with cardiovascular disease or high risk) and follow-up intervals.
- AHA/ACC and allied societies, 2025 guideline for the prevention, detection, evaluation and management of high blood pressure in adults.
- National Health Mission, Standard Treatment Guidelines — Hypertension: prevalence, the salt recommendation and the response-to-reading table.
- National Programme for Prevention and Control of Non-Communicable Diseases (NP-NCD) — population-based screening from age 30, screening and treatment figures as on February and July 2026, and the NCD portal.
- ICMR-INDIAB study, 2023 — hypertension prevalence of 35.5% and the age pattern.
- NFHS-5 (2019–21) — raised blood pressure in the 15–49 age group.

Prices and products

- Home monitors: Omron HEM-7090 recommended retail price ₹1,990 and HEM-7194T1-FLB ₹4,490 (Omron Healthcare India, November 2026); HEM-7124-IN ₹1,935 (Tata 1mg, November 2026); HEM-7383T1 ₹4,610 (Amazon.in, November 2026).
- Ambulatory BP monitoring: ₹1,800 in Kolkata to ₹4,000 in Noida and Greater Noida (Tata 1mg lab listing, last updated September 2026).
- Jan Aushadhi prices: telmisartan 40 mg ₹12 per 10 tablets; amlodipine 5 mg ₹4.50 per 10 tablets; telmisartan 40 + amlodipine 5 at ₹14.50 per 10 tablets and ₹22.69 per strip of 15 under PMBJP drug code 417 (scheme price data, June–October 2026).
- Branded comparisons: Telma 40 and Telma AM strip prices as listed in October 2026. Prices vary by pack and pharmacy; treat these as indicative.

What this book deliberately does not do

- It does not name a dose for you. Doses depend on your kidney function, your other medicines and your readings.

- It does not tell you to stop or start any prescription medicine.
- It does not diagnose white-coat or masked hypertension. It tells you how those are found and what they mean.
- It does not replace a clinical examination. High blood pressure is a diagnosis made by a clinician, using your numbers as evidence.

> **If one page of this book changes something for you**
>
> Make it the seven-day audit. A home average, taken properly, is the difference between guessing and knowing — and knowing is the only thing that has ever reliably moved this condition.

The Indian Blood Pressure Playbook 2026-27 — Digitalaikart. Written for Indian households, checked against Indian and WHO guidance, and priced so that it costs less than a month of branded tablets.
This document is educational material. It is not medical advice, not a diagnosis and not a prescription. Consult a registered medical practitioner for any decision about your health.